Centre universitaire de santé McGill

McGill University Health Centre

X HME MCH X HGM MGH X HRV RVH
X HNM MNH X ITM MCI X HL LH

FMU-3338

Ordonnances Médicales

Medical Orders

ALLERGIES _____

Poids / Weight (kg) _____ **Taille** / Height (cm) _____ **Surface corporelle / BSA (m^2)** _____

DATE-(AAYY/MM/JD) HEURE/TIME (00:00)	**ORDONNANCES DU PRESCRIPTEUR/** PRESCRIBER'S ORDERS **Le prescripteur doit indiquer son nom en lettres moulées, apposer sa signature et son N° de permis** Prescriber must write name in print letters, sign and write license No.	**Initiales de l'infirmier(ère) notées** Nurse's initials **noted**

PHARMACEUTICAL ORDERS – COMFORT CARE – ADULT

GENERAL INSTRUCTIONS

• **Notify at the 3rd PRN dose/24 h of the same medication, all indications combined**

Respiratory distress or bronchial congestion: ☐ See order "Pharm. orders – distress and bronchial congestion"

Pain / cough / dyspnea / tachypnea

SCHEDULED DOSES

☐ morphine _____ mg PO or _____ mg SC q4h (suggested min for opioid-naive patient: 2 mg PO / 1 mg SC)

☐ HYDROmorphone _____ mg PO or _____ mg SC q4h (suggested min for opioid-naive patient: 0.5 mg PO / 0.25 mg SC)

☐ long-acting opioid: _____ ☐ other: _____

BREAKTHROUGH DOSES – if pain, cough, dyspnea or RR greater than 28/min

Recommendation: use 50% of the q4h dose or 10% of the 24-h dose.

☐ morphine _____ mg PO or _____ mg SC q1h PRN

☐ HYDROmorphone _____ mg PO or _____ mg SC q1h PRN ☐ other: _____

Other symptoms

Fever (suspected) with discomfort: ☐ acetaminophen 650 mg PO/PR q4h PRN

Nausea and vomiting (choose only one medication)

☐ haloperidol _____ mg PO/SC q1h PRN (suggested minimum: 0.5 mg; avoid if seizure risk)

☐ ondansetron _____ mg PO/SC q6h PRN (suggested dose: 4 mg)

☐ dimenhyDRINATE _____ mg PO/SC q6h PRN (suggested minimum: 12.5 mg) ☐ Other: _____

Agitation: ☐ haloperidol _____ mg PO/SC q1h PRN (suggested minimum: 0.5 mg; avoid if seizure risk)

Anxiety / insomnia / spasms / myoclonus / tremor

☐ LORazepam _____ mg PO/SL/SC HS scheduled

☐ LORazepam _____ mg PO/SL/SC q4h PRN (suggested minimum: 0.5 mg) ☐ Other: _____

Seizures (if at risk: brain mets, epilepsy): ☐ LORazepam 2 mg SC × 1 dose if seizure, and notify the physician

Constipation: ☐ polyethylene glycol (Lax-A-Day®) ____ g PO ☐ daily or ☐ BID scheduled (hold if diarrhea)

Other:

☐ saliva substitute (Moi-Stir®) 1 to 2 oral sprays QID and PRN for dry mouth

☐ if symptom refractory to first-line treatment after 30 minutes: methotrimeprazine _____ mg PO/SC q2h PRN for ☐ anxiety ☐ insomnia ☐ agitation ☐ pain (suggested minimum: 2.5 mg)

☐ Other: _____

Prescriber (print name): _____ **Signature:** _____

License no.: _____ **Date** (yyyy/mm/dd): _____/____/____ **Time** (hh:mm): ____:____

Name: _____ **First name:** _____ **Chart #:** _____

Pharmacokinetic parameters

(adapted from the CMQ practice guide – *Les soins médicaux dans les derniers jours de la vie* – 2015)

Agent	Equivalence	Analgesic onset	Analgesic peak	Analgesic duration
Morphine	PO 5 mg SC 2.5 mg	PO 45–60 min SC 10–20 min	PO 1 h SC 0.5–1 h	PO 4–6 h SC 4–6 h
HYDROmorphone	PO 1 mg SC 0.5 mg	PO 30 min SC 10–20 min	PO 1 h SC 0.5–1 h	PO 3–5 h SC 3–5 h
OxyCODONE	PO 2.5 mg	PO 15–30 min	PO 0.5–1 h	PO 4–6 h

Fentanyl: transdermal 25 mcg/h = morphine 25 mg SC per 24 h. Analgesic onset 8 to 12 h after application; effect persists 8 to 12 h after removal.